NCCP 00309 · VERSION 6A

Your treatment, explained

Stage III non-small-cell lung cancer · concurrent chemoradiation · NCCP 00309 v6a · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Weekly carboplatin + paclitaxel with radiotherapy

7-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty, chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · smpc-paclitaxel

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- My treatment overall
- Your 7-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

My chemotherapy medicines

Chemotherapy is given alongside radiotherapy to treat stage III non-small-cell lung cancer when cisplatin is unsuitable. Your team explains the treatment aim and choices.

TAXANE CHEMOTHERAPY

Paclitaxel

Taxane chemotherapy



Disrupts cell division

Paclitaxel stabilises structures needed for cell division. It can also affect nerves, blood cells and healthy tissues. Premedicines help reduce infusion reactions.

Treatment reaches cells → Disrupts cell division → Cancer growth may slow

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What to watch for

Report numbness, tingling, muscle/joint pain or new breathing symptoms.

Sources: nccp · smpc-paclitaxel

PLATINUM CHEMOTHERAPY

Carboplatin

Platinum chemotherapy



Damages DNA

Carboplatin forms links in DNA that interfere with cell copying. Its dose uses kidney filtration and target exposure(AUC), not a fixed number of milligrams.

Treatment reaches cells → Damages DNA → Cancer growth may slow

What to watch for

Report fever, unusual bleeding, vomiting or allergy symptoms.

Sources: nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

My treatment overall

WEEKLY ×7 WITH RT

Seven weekly chemotherapy treatments

1 2 3 4 5 6 7

Both medicines are given by drip once every 7 days for seven weeks, alongside a separate radiotherapy course. The plan may change for side effects or test results. Further chemotherapy after radiotherapy is a separate decision.

SEPARATE APPOINTMENTS

Radiation course

Team-agreed

Radiation is given during the chemotherapy course. Its dose, number of fractions and precise timing are prescribed by radiation oncology. A chemotherapy delay does not automatically stop radiation.

REVIEW THEN NEXT PLAN

After chemoradiation

Review

Your team reviews recovery and response. NCCP describes optional separate two-cycle carboplatin/paclitaxel consolidation under00304; it is not automatically included and any next treatment needs a separate prescription.

The chemotherapy course is finite; radiotherapy timing and next treatment are confirmed separately.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

My tests and treatment checks

Required safety tests are distinct from team-agreed imaging and follow-up; no universal tumour-marker interval is specified.

Check	Before treatment	Treatment	Between visits	Next cycle / week	After treatment
Chemotherapy	Confirm aim and prescription	One visit each week	No additional routine doses	Seven weekly treatments	Next treatment only if agreed
Radiotherapy	Separate planning appointment	Alongside weekly treatment	Follow radiation appointments	RT team confirms ongoing plan	Review late symptoms
Tests	Blood count / kidneys / liver	Review required tests	Extra tests for symptoms	Bloods every week	Follow abnormal results

Check	Before treatment	Treatment	Between visits	Next cycle / week	After treatment
Symptoms / response	Baseline symptoms and contacts	Report new symptoms	Fever / breathlessness: urgent	Review tolerance	Individual scans / recovery

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00309 v6a · official regimen

THE WHOLE TREATMENT JOURNEY

Weekly carboplatin + paclitaxel with radiotherapy: treatment and checks

Stage III non-small-cell lung cancer · concurrent chemoradiation

Both medicines are given by drip once every 7 days for seven weeks, alongside a separate radiotherapy course. The plan may change for side effects or test results. Further chemotherapy after radiotherapy is a separate decision.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment	Between visits	Next cycle / week	After treatment
Chemotherapy NCCP 00309	Confirm aim and prescription	One visit each week	No additional routine doses	Seven weekly treatments	Next treatment only if agreed
Radiotherapy NCCP 00309	Separate planning appointment	Alongside weekly treatment	Follow radiation appointments	RT team confirms ongoing plan	Review late symptoms
Tests NCCP 00309	Blood count / kidneys / liver	Review required tests	Extra tests for symptoms	Bloods every week	Follow abnormal results
Symptoms / response NCCP 00309	Baseline symptoms and contacts	Report new symptoms	Fever / breathlessness: urgent	Review tolerance	Individual scans / recovery

What each check is for

1

Prepare

Before first dose

Chemotherapy is given alongside radiotherapy to treat stage III non-small-cell lung cancer when cisplatin is unsuitable. Your team explains the treatment aim and choices.

Ask / act: Confirm prescription, radiation plan and contacts.

Sources: NCCP 00309

2

Monitor

Each week

Check fitness and tolerability

Ask / act: Report symptoms and confirm test results before dosing.

Sources: NCCP 00309

3

Review

After course / when clinically indicated

Assess response and recovery

Ask / act: Confirm the next plan and persistent symptoms.

Sources: NCCP 00309

At the next review, ask

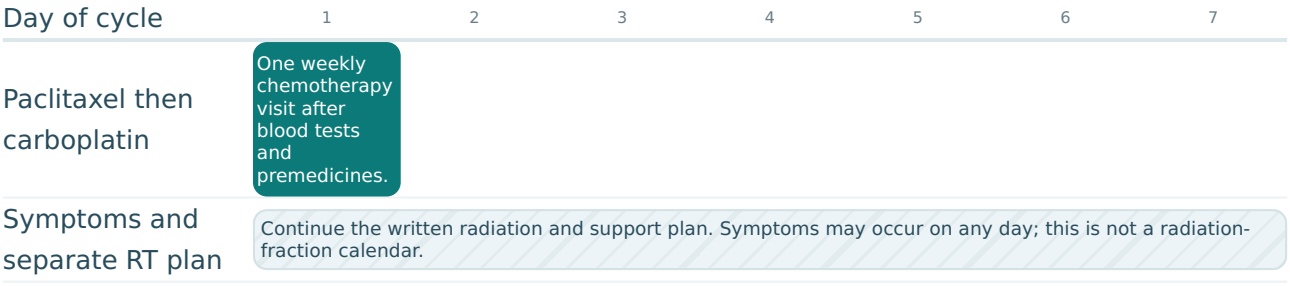
- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Both medicines are given by drip once every 7 days for seven weeks, alongside a separate radiotherapy course. The plan may change for side effects or test results. Further chemotherapy after radiotherapy is a separate decision.



Relative schedule, not personal dates: Both medicines are given by drip once every 7 days for seven weeks, alongside a separate radiotherapy course. The plan may change for side effects or test results. Further chemotherapy after radiotherapy is a separate decision.

No fixed symptom day: Sickness, fatigue, low counts, swallowing problems or breathing symptoms can occur between visits and after treatment. Do not wait for a calendar window to seek advice.

Radiation is a separate schedule: Chemotherapy Day 1/Day 2/Day 3 or weekly counts are not radiation fractions. The radiation oncology team supplies the exact RT timetable.

Why these bands stay broad: Reference cycle without patient dates. RT visits and dose are separately prescribed; the symptom band is not a nadir prediction. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00309 v6a · official regimen ↗

There are no guaranteed “safe days”. Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty, chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

Read the whole cycle as text

Day 1: Weekly treatment day

Blood results and symptoms are reviewed. Paclitaxel comes first, followed by carboplatin. Tell staff immediately about flushing, breathing trouble, pain or swelling during infusion.

Day 2–3: Support at home

Use the anti-sickness medicines prescribed for these days. Radiation visits may continue on their own schedule. Call for fever, poor intake, vomiting or new breathing symptoms.

Day 4–7: Before the next weekly dose

Keep track of swallowing, fatigue, numbness and muscle/joint symptoms. Arrange the next weekly blood tests. Report problems before the next dose; do not wait if acutely unwell.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Confirm your plan

Paclitaxel is infused first over one hour, then carboplatin over 30 minutes, plus checks and premedicines. The consultant may prescribe a lower paclitaxel dose. RT appointments are separate and are not counted as chemotherapy doses.



STEP 2

Keep the contact card

The team supplies the daytime and out-of-hours numbers. Keep the card with you and show it when seeking medical care.



STEP 3

Two linked treatment teams

Confirm which team to contact for radiation or chemotherapy changes. Report swallowing problems early and ask about transport, nutrition and fatigue support.

Open practical treatment guide · **Printable practical guide**

Sources: NCCP 00309 v6a · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Weekly medicines before treatment

Paclitaxel needs premedicines to reduce allergic reactions. The first week and later weeks can differ. Your team also provides anti-sickness medicines for carboplatin; do not double up steroid doses.

2 Eating, swallowing and breathing

Chest radiation can make swallowing sore. Ask early for pain relief and nutrition support. New breathlessness, cough or inability to drink needs urgent advice, even after the course finishes.

3 Nerves and aches

Tell the team about numbness, tingling or muscle/joint pain before each weekly visit. Follow the agreed pain-relief plan and check before starting medicines or supplements.

Sources: nccp · ncis

Why chemotherapy and radiotherapy are combined

Studies support combining systemic treatment with radiation in the relevant lung-cancer setting.

The evidence comes from specific treatment schedules and patient groups. Your team explains how it applies to your situation; study averages cannot predict an individual outcome.

Sources: trial

PREPARE FOR THE CONVERSATION

Questions worth asking

Which drugs and routes are prescribed for me?

Confirm the exact drug names, routes and interval against your written prescription. The team must explain any change from this reference plan.

How do my radiation appointments fit around chemotherapy?

Radiation has its own calendar. Ask for the combined plan and whom to contact if a chemotherapy or radiation visit changes.

Which blood tests are required before the next dose?

Use the checks table as a prompt. Your team confirms which tests are needed, where they are done and who reviews the results before treatment.

Who do I call during the day and overnight?

Ask the team to replace the example numbers on your card with the correct daytime, overnight and backup contact routes before you leave.

When will recovery and response be reviewed?

Agree the follow-up appointments and any scans with your team. This regimen does not set a universal scan or tumour-marker timetable.

> **Try explaining the plan back:** Explain back your chemotherapy days, how radiation is scheduled separately, the tests before treatment and whom you will call if unwell.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00309 v6a. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00309 v6a · official regimen
- NCCP NCIS medical-oncology antiemetics v8
- Paclitaxel manufacturer product information
- Choy et al.1998 · phase II weekly chemoradiation

SACTCheck · Developed by Dr Paul O’Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://assets.hse.ie/media/documents/309_CARBOplatinAUC2_and_PACLitaxel50_weekly_with_RT.pdf

https://assets.hse.ie/media/documents/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://www.medicines.org.uk/emc/product/3891/smpc

https://pubmed.ncbi.nlm.nih.gov/9779707/